



DEPARTMENT OF HISTOPATHOLOGY
Research Block-A, 5th Floor, PGIMER, Chandigarh
SURGICAL PATHOLOGY REPORT

Name	V K Gurg	Age/Sex	66 Yr/M	Biopsy No.	S-30929/2024
Clinician	-	Request Date	30/10/2024	Cr.No	201405136462
Department	Clinical Hematology & Medical Oncology-Hematology Clinic	Report Date	04/11/2024		

Clinical Diagnosis :

- Multiple Myeloma

Gross :

Received 2 slides and 1 block for review labelled as 734/2024.

Micro :

- Slides and section examined from block show native renal biopsy which is evaluated using H & E, and MT stains under light microscopy. Per level of the sections shows single linear core of renal cortex comprising of a maximum of 10 glomeruli. Out of which 1 glomerulus is globally sclerosed. Rest of the glomeruli show ischemic changes with underlying tuft showing reduplication of basement membrane, mesangial expansion with foaminess and endothelial swelling indicating subacute thrombotic microangiopathy changes.

There is moderate to severe acute tubular injury and presence of vacuolations within tubular epithelial cells accompanied by moderate increase in lymphomononuclear inflammation admixed with an occasional scattered eosinophils and collection of histiocytes forming granuloma at places. In addition, there is diffuse interstitial edema and early fibrosis with tubular atrophy of 25-30% of the core.

Most of the arteries and arterioles show moderate to severe fibrointimal thickening, medial hypertrophy with mucinoid cystic changes and moderate to severe sclerosis causing obliteration of lumen indicating subacute to chronic thrombotic microangiopathy changes.

DIF for Kappa and Lambda performed on pronase treated paraffin section and show 1-2+ deposits in tubular cast. There is no light chain restriction.

EM sample : Not sent

In this 66 years old male, known case of MM , post Carfilzomib induced TMA,

This native kidney biopsy (slides and block for review) shows features of 1) Evidence of Subacute glomerular and subacute to chronic vascular thrombotic microangiopathy. 2) Moderate to severe Acute tubular injury. 3) Toxic Tubulopathy

Validated On	14-11-24 01:30 PM	Validated By	Ritambhara Nada(Professor)
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Diagnosis :

Kidney (native biopsy) (Slides and block for review) Subacute to chronic glomerular and vascular- Thrombotic microangiopathy

Kidney (native biopsy) (Slides and block for review) Moderate to severe- Acute tubular injury

Kidney (native biopsy) (Slides and block for review) Toxic- Tubulopathy

Immunohistology :

Supplementary Report :

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Remarks :

IMA/AN

Ritambhra Nada / -

Validated On

14-11-24 01:30 PM

Validated By

Ritambhra Nada(Professor)